



Closure of Entry and Access Doors in the Mental Health Unit Due to Clinical Needs Procedure

1. Applicability

Site	Operational Area	Applicable to
Armadale Mental Health Service	All areas	All workforce groups

2. Purpose

The purpose of this policy is to ensure a safe and appropriate environment is maintained for staff, patients and visitors. Policy conforms to the intent of the Western Australian [Mental Health Act 2014](#) and [Charter of Mental Health Care Principles](#).

To ensure this safe environment for staff, patients and visitors the main entry door into the adult inpatient Moodjar unit, the Karri Courtyard access door, and the corridor access door, hereafter referred to as “the doors”, are closed daily at 1700hrs. This includes weekends and public holidays.

In accordance with the [EMHS Mental Health Patient Flow and Bed Capacity Framework](#) capacity making strategies for BRAG Amber and above, there may be a requirement for the doors to be closed outside of these times to assist in providing a safe environment by contributing to the mitigation of clinical risk or high levels of patient acuity on the ward.

This action should only be undertaken following consideration of less restrictive options such as increasing visual observations, placing a patient on a special or temporarily transferring the patient to the closed ward.

3. Procedure

Closure of the doors includes the ensuring all doors inclusive of the entrances to the ward and all shared courtyards are closed and require staff access to be opened.

3.1 Authorisation of Closure of the doors

1. The Nurse Unit Manager (NUM), Associate Nurse Unit Manager (ANUM), After Hours Clinical Nurse Specialist (AHCNS) or the Shift Coordinator in consultation with the treating Medical Officer or treating Consultant must first sanction the closure of the doors. If closure of the door is initiated by the Shift Coordinator, the Shift Coordinator will notify the NUM, ANUM or AHCNS, and the treating Medical Officer will notify the treating Consultant as soon as practicable.
2. If outside business hours the After-Hours Clinical Nurse Specialist (AHCNS) or the Shift Coordinator in consultation with the Duty Medical Officer or On-call Consultant must first sanction the closure of the doors. If closure of the doors is initiated by the Shift Coordinator, the Shift Coordinator will notify the AHCNS, and the Duty Medical Officer will notify the On-call Consultant as soon as practicable.

3.2 Process for Closure of the doors

1. The Allocated nurse commences 15/60 Safe and supportive observations as per [AKG Safe and Supportive Observations: Procedure MEN-PRO-0310-16](#) which remain for the duration of the doors being closed. The Allocated nurse also updates i.Clinical manager with the new SASO.
2. All consumers are to be advised that the doors are required to be closed for clinical reasons and that accessibility should be via their allocated nurse. Signage should be placed on the external and internal of all affected doors, advising that the doors are to be secure on exit and entry. The closure of the doors does not impede the access to and from the ward for voluntary patients as identified in their care plan and in accordance with the [AKG Granting and Management of leave for AKG Mental Health Inpatient Unit Guideline](#).
 - 2.1. The shift coordinator advises the ward clerk, if present, to affix signage on the external and internal aspects of the main entry doors, advising that the doors are to be secured on exit and entry and advise patients and visitors that staff can be contacted for facilitating access or exit. If not present the Shift coordinator is to allocate a nurse to complete this task.
 - 2.2. The shift coordinator affixes signage on the external and internal of the Karri courtyard and corridor entry doors, advising that the doors are to be secured on exit and entry.
3. The treating team Consultant conducts an in-person review of the patient within two hours of the closure of the doors.
 - 3.1. The on-call consultant is to conduct the review as soon as is practicable if afterhours.
4. The treating team Consultant or On-call Consultant updates the DMR identifying the rationale for the doors being closed and reviews the Patients Risk Assessment and Management Plan, updating as necessary.
5. The Shift coordinator updates the ward report to reflect the doors being closed. This will ensure the clinical team discuss continuation of closure of the doors and potential mitigating strategies to manage behaviours of concern at the AKG Mental Health huddle.

3.3 Review of Closure of the doors

1. The treating team Consultant or On-call Consultant are to review the patient every 24 hours that the doors remain closed, updating the DMR with rationale for continued closure and alternative management plans explored.
2. In the event there is a breach of security, the Shift Coordinator will complete a head count of patients. If the patient at risk is noted to be missing, the Shift Coordinator will implement the [WA Department of Health Missing Person Policy](#) in conjunction with the [EMHS Missing or Suspected Missing Mental Health Patients/Consumers](#)

In the event of the doors being closed for four patients, the NUM, ANUM or AHCNS are to facilitate an 'Acuity Nurse" due to increased level of Safe and Supportive Observations and to help reduce the clinical risk on the ward. When there is a request to go above 6 patients requiring doors closed, the NUM, ANUM or AHCNS on duty must escalate this, in business hours to the Nurse Coordinator of Mental Health, who will then escalate it to the Head of

Service and the Service Director of Mental Health. After hours, escalation should be directed to SROC.

4. Roles and responsibilities

The following outlines the specific roles and responsibilities relevant to this procedure:

Role	Responsibility
Service Director Mental Health	The Service Director has overall responsibility for ensuring that services are delivered in accordance with this
HoD Psychiatry	The Head of Psychiatry has overall responsibility for ensuring that medical staff are given orientation and education on the use of this procedure and to ensure that services are delivered in accordance with this procedure.
NUM and ANUM	To provide orientation and education to relevant AKG clinicians and staff on the use of this procedure.
Mental Health Clerical coordinator	To provide orientation and education to relevant AKG clerical staff on the use of this procedure.
AKG staff	All AKG mental health staff are required to work within this procedure.

5. Compliance monitoring

Compliance against this procedure will be monitored by the Mental Health Safety and Quality committee.

Compliance monitoring activities relevant to this procedure include:

- A record of occasions when the doors have been closed due to clinical needs will be collated by the NUM Moodjar and submitted to the Nursing Coordinator Mental Health, monthly, and to be tabled at the Armadale Mental Health Service, Department Meeting: Safety and Quality.
- Monitoring clinical incidents reported via Datix Clinical Incident Management System (CIMS).

6. Policy context

The following documents are required to give effect to this procedure:

- Mental Health Act 2014*
- Health Department of Western Australia (HDWA) [Clinical Incident Management Policy MP 0122/19](#)
- Chief Psychiatrist [Reporting of Notifiable Incidents to the Chief Psychiatrist](#)
- East Metropolitan Health Service (EMHS) [Missing or Suspected Missing Mental Health Patients / Consumers EMHS: 47](#)
- [AKG Safe and Supportive Observations: Procedure MEN-PRO-0310-16](#)
- [EMHS Mental Health Patient Flow and Bed Capacity Framework](#)

7. Supporting information

The following documents support the implementation of this procedure:

- [AKG Granting and Management of leave for AKG Mental Health Inpatient Unit Guideline.](#)
- [Duress System, Personal – Leschen Unit MEN-PRO-0138-13](#)
- [Afterhours Facilities Issues AKG-PRO-0015-17](#)
- [Charter of Mental Health Care Principles](#)
- Education and training resources

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 1 – Action 1.29 Safe environment
- Standard 5 – Action 5.33 Predicting, preventing and managing aggression and violence

National Standards for Mental Health Services (2010):

- Standard 1 – Criteria 1.1

9. Glossary

The following definitions are relevant to this procedure:

Term	Definition
'The Doors'	The term 'the doors'; includes: Moodjar main entry door, Karri Courtyard access door and corridor access door

10. Document contact

Enquiries relating to this procedure may be directed to:

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 Department: Mental Health Services
 Email: Rijo.Joseph@health.wa.gov.au

11. Review

This procedure will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
5	08/07/2024	08/07/2027	

12. Authorisation

This procedure has been authorised and issued by:

Authorised by	Karen Elliott – Service Director Mental Health
Authorisation date	08/07/2024
Published date	09/07/2024